Engaging reintegrating military families in a web-enhanced parenting program: After Deployment, Adaptive Parenting Tools (ADAPT)

Sheila Hanson, Ph.D., Keri Pinna, Ph.D., Dustin Brockberg, B.S., & Abigail H. Gewirtz, Ph.D.

Department of Family Social Science, University of Minnesota

Background

Families and deployments

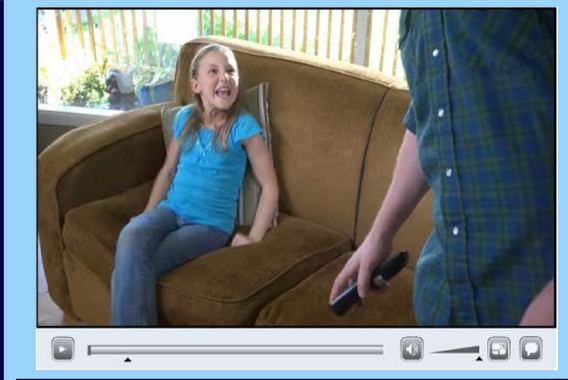
It is estimated that more than three million family members have been affected by the deployment of more than two million troops to the wars in Iraq and Afghanistan. Fifty percent of military personnel are married, and forty percent have children; 63% of military families live off military bases in more than 4000 communities across the USA (Department of Defense, 2011). The wars in Iraq and Afghanistan also have resulted in deployments of unprecedented numbers of 'civilian soldiers' – those in the National Guard and Reserves (NG/R). On average, NG/R personnel are older, and more likely to be partnered and parenting, than 'regular' active duty military personnel. The high operational tempo of the conflicts in Iraq and Afghanistan also has resulted in more frequent and longer deployments of these personnel. For example, average deployment of Army National Guard personnel since 2001 has been 2.2 times, each of an average length of 12 months. Other services (e.g. Air National Guard) see shorter but far more frequent deployments.

Rationale for ADAPT

We focus on parenting as a crucial mechanism for child and family functioning following deployment. We conceptualize deployment as a family stressor for two reasons: the separation inherent in a parent's deployment, and stress associated with the residual impact of exposure to combat on parental functioning during reintegration. Drawing from family stress research, and a social interaction learning framework, we provide a theoretical/conceptual rationale for the After Deployment, Adaptive Parenting Tools (ADAPT) program, a web-enhanced, group-based parent training intervention currently being evaluated in an NIH-funded randomized controlled trial. We provide preliminary data on the feasibility and acceptability of the program, as well as data on dosage (physical and online participation/engagement) for the first cohort of families participating in the ADAPT program.

ADAPT study and participants

The ADAPT parenting program is being tested in a randomized effectiveness trial funded by the National Institutes of Health (R01-DA030114). Families are randomly assigned to participate in the experimental group/ADAPT program (60% of total sample, final $N = 240$) or to services-as-usual (i.e. print, online, and email parenting resources; 40% of total sample, final $N = 160$). Families participate in four assessment interviews conducted over a two-year period to investigate the effectiveness of the parenting program on parenting, child, and parent outcomes. We report information for the first 42 families assigned to the intervention condition/ADAPT group program.





ADAPT Conceptual Framework

Applying the social interaction learning model to intervention, Forgatch & DeGarmo (1999), Dishion & Andrews (1995), Chamberlain et al. (2008), and others developed parent training interventions to buffer parenting in stressful situations with the goal of improving child outcomes. ADAPT modified evidence-based parenting interventions for military families. The adapted curriculum includes mindfulness practices to address emotional awareness. To our knowledge, this is the first empirically-supported parent training program targeting school age children that is modified specifically for National Guard and Reserve military families with a web-based supplement.

Measures

Feasibility on web-enhanced ADAPT

- weekly attendance by parents and families
- completing home practice assignments
- usage of online tools

Parent feedback

20-item participant feedback survey. 5-point Likert scale ratings (1=not at all to 5=very much):

Participant satisfaction (agreed with ideas, info today was helpful, enjoyed group).

Positive leader qualities (leader encouraged participation, leader understands me, liked group leader).

Active participation (did homework assignment, paid attention, actively participated, open to new info).

Negative mood (felt angry or irritable, felt sad/depressed).

Acceptable content (home practice was helpful, home practice was hard, I was successful with home practice, my children responded well, assignment fits well with my family life).

Positive emotional climate (felt accepted by group members, pleasantly humorous things happened, felt put down by someone [reversed]).

Control variables

Demographic variables including marital status, income, education, employment, minority status, ethnicity, and number of children in the household.

Results

Attendance, home practice, and online tools

Families who attended more sessions also completed more HPAs ($r = .85, p < .01$). Family attendance and completion of HPAs were significantly correlated with total use of online tools ($r = .35$, $p = .05$ and $r = .43$, $p = .01$). At the parent level, parent attendance and HPA completion were correlated at $r = .90, p < .01$. Both were correlated with total number of online components completed ($r = .36$, $p = .01$ and $r = .41$, $p < .01$, respectively). Parent attendance and HPA completion related to use of online types of components in similar ways to the family level.

Online Tool Usage	Average Proportion (SD)
Videos	.30 (.35)
Mindfulness Exercises	.19 (.29)
Summaries	.33 (.40)
Handouts	.25 (.35)
Knowledge Checks	.32 (.41)

Table 1. Average Proportion of Online Tool Types Used by Families

Summary and Discussion

Data from the first two cohorts of ADAPT intervention participants suggests that the program is both feasible and acceptable. Seventy-eight percent of families invited to participate in the intervention came to at least one session. Once a family attended at least one session, average participation was extremely high for a parenting group (79% attended at least seven out of fourteen, weekly, 2-hour sessions).

Our rationale for developing the web tools was to provide additional parenting tools that did not require face-to-face participation; we were thus surprised to see web participation significantly associated with group participation.

Participation in different aspects of the program was associated – that is, families who attended more in-person group sessions, also used the web tools more frequently. The exception was access to the mindfulness exercises. We speculate that motivation to access the online mindfulness exercises may more likely be associated with an individual's (or family's) readiness to practice strategies that are qualitatively quite different from parenting strategies. It is possible that baseline mindfulness or other related variables (e.g. positive attitudes towards mindfulness and meditation) may be most associated with an individual's use of these exercises, rather than general motivation to address parenting. Some parent participants expressed questions about how mindfulness was relevant to parenting. It is not immediately obvious, until parents try it, why a 10-minute mindfulness practice would improve parenting compared with practice at giving effective directions.

On average, participants gave high satisfaction ratings for every group session (on average 3.3, on a 1-4 scale, or 'quite satisfied'), and ratings did not differ by gender (or other demographic variables). In a regression equation examining predictors of participation, overall participant feedback was associated with home practice assignment completion only (and not group or web participation), controlling for demographics. This is not surprising; we would expect that parents who perceived the program to be more acceptable might invest more in ADAPT.

We are hopeful that the combination of high satisfaction and high participation in the program will prime participants for positive behavior change resulting in improved parenting and child outcomes for military families. As follow-up data become available, we will explore the extent to which greater participation in ADAPT predicts improvements in parenting and child outcomes for military families.